

HIV: ROUTINE CARE

Assess the patient with HIV

Assess	When to assess	Note	
Symptoms	Every visit	Manage patient’s symptoms as on symptom pages. If genital discharge/ulcer or partner has been treated for an STI in past 8 weeks, manage for STI ↗ 49.	
TB	Every visit	Test for TB (send 1 sputum sample for TB NAAT): at HIV diagnosis, yearly with VL tests, and if any TB symptoms develop (cough, weight loss, night sweats or fever) ↗ 92.	
Adherence	Every visit	Check record of patient's adherence to treatment and facility visits. If difficulty with adherence, give adherence support ↗ 173.	
Side effects	Every visit	- Ask about side effects from ART ↗ 116, TB preventive treament (TPT) ↗ 89, co-trimoxazole preventive therapy (CPT) and fluconazole ↗ 113. Manage promptly. Discuss if unsure. - If suspected adverse drug reaction, report via the MedSafety app or fill form and submit to pharmacist, or email to adr@sahpra.org.za.	
Depression	Every visit	In the past month, has patient: 1) felt down, depressed, hopeless or 2) felt little interest or pleasure in doing things? If yes to either ↗ 143.	
Alcohol/drug use	Every visit	In the past year, has patient: 1) drunk ≥ 4 drinks¹/session, 2) used illegal drugs or 3) misused prescription or over-the-counter medications? If yes to any ↗ 142.	
Sexual health	Every visit	If risky sexual behaviour: new or multiple partner/s, uses condoms unreliably, has sex under influence of alcohol/drugs, give safe sex advice.	
Family planning	Every visit	- If woman of child bearing potential, sexually active and not on reliable contraception, exclude pregnancy ↗ 157. - Assess patient's contraceptive needs. Advise reliable contraception (condoms <i>plus</i> IUD, subdermal implant, injectable or sterilisation) ↗ 154. - Reassure that there are no interactions with DTG. If on other ART regimen, assess eligibility to switch to DTG ↗ 117 or adjust contraception ↗ 118. - If planning pregnancy: start folate 5mg daily and advise to defer pregnancy until viral load < 50. Check for syphilis ↗ 53.	
Vertical transmission prevention (VTP)	Pregnant/ breastfeeding	- If not on ART, start ART same day. If pregnant, also give antenatal care ↗ 159. - If breastfeeding, check that HIV-exposed infant has received correct PEP and PCR results ↗ 168.	
Palliative care	If deteriorating	If failing 3rd line ART and deteriorating, also give palliative care ↗ 170.	
Weight	Every visit	- At diagnosis, measure height and weight to calculate BMI. BMI = weight (kg) ÷ height (m) ÷ height (m): - If BMI < 18.5, refer for nutritional support. If BMI ≥ 25, assess CVD risk below. - If weight loss ≥ 5% of body weight in 4 weeks ↗ 23.	
Chronic conditions and CVD risk	At diagnosis	- If known hypertension, check control ↗ 133. If not known with hypertension, check BP: if ≥ 140/90 ↗ 132. - If known diabetes, check glucose control and adjust doses of metformin if taking together with dolutegravir ↗ 130. If not known with diabetes, check glucose ↗ 17. - If known with epilepsy, check seizure control and for possible medication interactions ↗ 149. - Assess CVD risk ↗ 127. If CVD risk > 20% or known CVD², and on LPVr or ATV/r, switch to DTG ↗ 117. If unable to switch to DTG, switch to ATVr and change simvastatin to atorvastatin 10mg at night.	
WHO Clinical Stage	Every visit to check if stage has worsened	- Check weight, mouth, skin, previous and current problems. Once on ART, the aim is for patient to be WHO Clinical Stage 1. - Use WHO Clinical Stage to decide when to start co-trimoxazole ↗ 113. If not on ART, use most advanced stage even if recovered. If on ART, use stage done at this visit.	
WHO Clinical Stage 1	WHO Clinical Stage 2	WHO Clinical Stage 3	WHO Clinical Stage 4
- No symptoms - Persistent painless swollen glands	- Recurrent sinusitis, tonsillitis, otitis media, pharyngitis - Papular pruritic eruption (PPE) - Fungal nail infections - Herpes zoster (shingles) - Recurrent mouth ulcers - Angular cheilitis/stomatitis - Unexplained weight loss < 10% body weight	- Pulmonary TB within past year - Oral candida - Oral hairy leukoplakia - Unexplained weight loss ≥ 10% body weight or BMI < 18.5 - Unexplained diarrhoea > 1 month - Unexplained fever > 1 month - Severe bacterial infections (pneumonia, meningitis) - Unexplained anaemia < 8, neutropaenia < 0.5 or chronic thrombocytopaenia < 50	- Extrapulmonary TB within past year - Weight loss ≥ 10% and diarrhoea or fever > 1 month - Pneumocystis pneumonia (PJP) - Recurrent severe bacterial pneumonia - Herpes simplex of mouth or genital area > 1 month - Oesophageal candida - Kaposi’s sarcoma, lymphoma, invasive cervical cancer - Cytomegalovirus infection - Toxoplasmosis - HIV-associated dementia, encephalopathy - Cryptococcal disease (including meningitis) <i>Cryptosporidium</i> or <i>Isospora belli</i> diarrhoea

Continue to assess the patient with HIV → 112.

¹One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer. ²Cardiovascular disease (CVD) includes ischaemic heart disease, peripheral vascular disease and stroke/TIA.